

MONARCH GENERAL HOSPITAL

600 Secure Lane, McLean, VA 22102 | (703)555-6607 | www.monarchgh.org

DISCHARGE SUMMARY

Patient Name: Hana Nakamura	DOB: March 31, 1984 (Age 41)
MRN: MRN-9642649	Gender: Male
Admission Date: July 29, 2025	Discharge Date: August 19, 2025 (21 days)
Attending Physician: Dr. Astrid Lindqvist, MD — Pulmonology / MICU	Insurance: Cigna OAP #CIG-G7-77777
ICU Admission: YES — MICU/SICU	ICU Days: 5 of 21 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute Decompensated Heart Failure — EF 18%	I50.9	Acute / Critical
Severe COPD — FEV1 28% Predicted	J44.1	Chronic / Complicating
Malnutrition — Severe Protein-Energy	E43	Chronic / Complicating

HOSPITAL COURSE

Hana, a 41-year-old male with a history of Severe COPD — FEV1 28% Predicted and Malnutrition — Severe Protein-Energy, presented with acute hypoxic respiratory failure requiring emergent intubation and mechanical ventilation in the ED. Chest CT confirmed bilateral ground-glass opacities consistent with ARDS superimposed on community-acquired pneumonia. The patient was managed with lung-protective ventilation (tidal volume 6 mL/kg IBW, PEEP 12 cmH2O) and prone positioning for 16 hours per day over 3 days. Steroids were initiated per RECOVERY protocol. Tracheostomy was performed on day 10 due to failure to wean. The patient was gradually decannulated by day 18 and transferred to a long-term acute care facility for ventilator weaning rehabilitation.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
90/68 mmHg	104 bpm	93% (on 2L O2)	99.6°F	23/min	86 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
G-CSF (Filgrastim)	480 mcg SC	Once daily	Until ANC > 1.0 x10 ³ /μL
Furosemide IV	80 mg	Twice daily	For diuresis — reassess at 1 week
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Metronidazole	500 mg	Every 8 hours	7-day course

FOLLOW-UP PLAN

URGENT: Pulmonology / MICU: Dr. Astrid Lindqvist, MD — within 72 hours of discharge (August 22, 2025).

Primary Care: Within 1 week (August 26, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.
Inpatient rehabilitation / LTACH: Transfer arranged — starts September 01, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Astrid Lindqvist, MD — Pulmonology / MICU | License #: VA-MD-87901 | Date: August 19, 2025

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